CaseNote Lab

OET Writing workbook for nurses

Free sample case · full pack (24 cases + app) at kitwisestudio.github.io/casenote-lab

How to use this workbook: read the case notes for 5 minutes without writing, then write your letter in 40 minutes on the lined page (body about 180–200 words). Afterwards, type it into the CaseNote Lab app to check which key notes you used, then compare with the model letter at the back.

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T06	Non-healing leg ulcer – referral to tissue viability nurse	Referral	medium

T06 · Non-healing leg ulcer – referral to tissue viability nurse

Community nursing · Referral · medium · Today's date: 12 March 2026

Writing task. Using the information in the case notes, write a letter of referral to Ms Fiona Grant, Tissue Viability Nurse Specialist at Lakeside Community Health Services, requesting specialist assessment of Mrs Mallory's leg ulcer. Address the letter to Ms Fiona Grant, Tissue Viability Nurse Specialist, Lakeside Community Health Services, Lakeside Clinic, 9 Marina Way, Lakeside. In your answer: expand the relevant notes into complete sentences; do not use note form; use letter format. The body of the letter should be approximately 180–200 words.

SOCIAL BACKGROUND

- Lives with husband (80, early dementia) – she is his main carer
- Mobile with walking stick; daughter can drive her to clinic
- Volunteers at charity shop 1 day/wk

MEDICAL HISTORY

- Previous DVT L leg 2009
- T2DM – metformin 500mg bd; HbA1c 55 mmol/mol
- Hypertension – amlodipine 5mg od
- Hysterectomy 1990
- Cataract removal R eye 2022
- Allergy: Elastoplast (rash)
- BMI 32
- Private podiatry monthly – toenail cutting

WOUND HISTORY

- Venous leg ulcer L medial gaiter – first noted 01/01/2026
- Cause: knocked leg on car door
- Ulcer ↑ in size despite 10 wks treatment
- ABPI 0.78 (20/01/2026)
- Reduced compression bandaging since 01/02 – tolerating

CURRENT ASSESSMENT

- Size 6.5 x 4cm (was 4 x 3cm on 01/02)
- Wound bed: 60% slough, 40% granulation
- Heavy serous exudate – daily strike-through; maceration of surrounding skin
- Haemosiderin staining, dry scaly skin to lower leg
- No odour; swab 02/03 – normal skin flora
- Dressing: Aquacel Extra (hydrofiber) + absorbent pad, 3x/wk
- Pain 6/10 at dressing changes – paracetamol 1g qds
- Sleeps in armchair at night due to pain – legs dependent
- Anxious ulcer will never heal

PLAN

- Request TVN assessment: dressing & compression review; ?vascular referral / duplex scan
- District nurse visits Mon/Wed/Fri

Your letter

Your letter

Model letters

Try each task yourself before reading its model letter.

T06 · Non-healing leg ulcer – referral to tissue viability nurse

Ms Fiona Grant
Tissue Viability Nurse Specialist
Lakeside Community Health Services
Lakeside Clinic
9 Marina Way
Lakeside

12 March 2026

Dear Ms Grant,

Re: Mrs Irene Mallory, DOB 21/09/1947

I am writing to refer Mrs Mallory for specialist assessment of a venous leg ulcer on her left medial gaiter area, which has deteriorated despite 10 weeks of treatment.

The ulcer has increased in size to 6.5 x 4cm. The wound bed is 60% slough and 40% granulation tissue. Heavy serous exudate causes daily strike-through, and the surrounding skin is macerated. It is currently dressed with Aquacel Extra hydrofiber and an absorbent pad three times weekly. Mrs Mallory reports pain of 6/10 at dressing changes despite taking paracetamol 1g four times daily.

Her ABPI was 0.78 in January, and reduced compression bandaging has been applied since 1 February, which she tolerates. She has a history of a left leg DVT in 2009 and also has type 2 diabetes. Please note that she is allergic to Elastoplast. Because of pain, she sleeps in an armchair at night, so her legs remain dependent.

I would be grateful if you could review her dressing and compression regimen and consider whether a vascular referral is required.

Please do not hesitate to contact me if you require further information.

Yours sincerely,

Community Staff Nurse
Lakeside District Nursing Team

WHY IT IS WRITTEN THIS WAY

- The first sentence states the request (specialist assessment) and the main problem (a deteriorating ulcer despite 10 weeks of treatment).
- The wound is described in a logical order: size, wound bed, exudate and skin, current dressing, pain.
- Vascular status (ABPI) is placed next to the compression details because the TVN will use them together to decide treatment.
- The Elastoplast allergy and sleeping in a chair are short notes but essential: they affect dressing choice and oedema.
- Old unrelated surgery, charity work and the DN visiting days are omitted. The cause (car door) is optional background.

- The final paragraph repeats the request with specific actions, which helps the reader know exactly what is expected.